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Project #53: Clinical Decision Unit (CDU)

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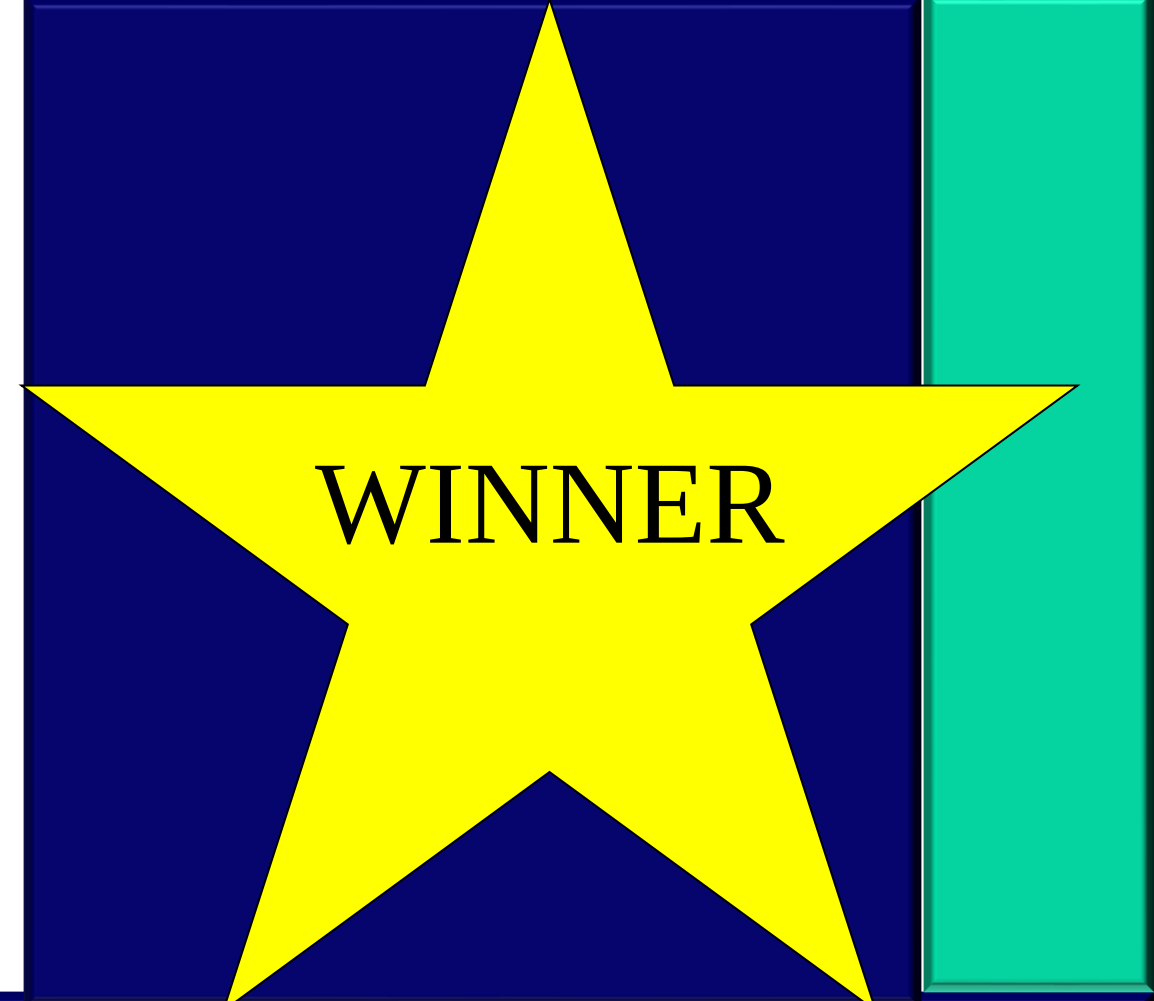
Authors

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Clinical Decision Unit (CDU)

Team Members:

Anna Leszczynski RN, BSN-ED ACM, Elizabeth Ashley RN, MBA-ED Manager, Kristina Cross RN, BSN-Clinical Nursing Coordinator, Wendy Butterfield RN, BSN-ED Educator, Dr. Ghazwan Atto MD-CDU provider, Dr. Ghassan Atto MD – CDU Provider, Toni Silas RN MSN-Nursing Administrator, Dr. Plemmons MD, FACEP – Chair and Medical Director of the Department of Emergency Medicine, Dr. Spencer DO, FACEP – Assistant Medical Director of Emergency Medicine, Director of Quality

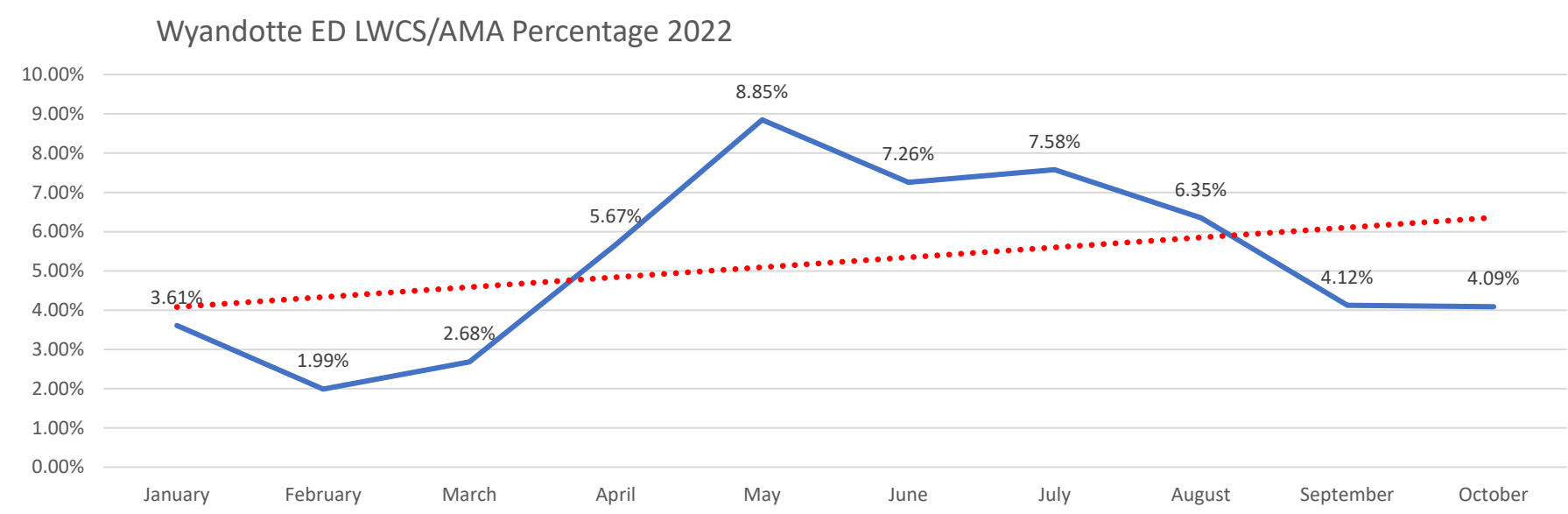
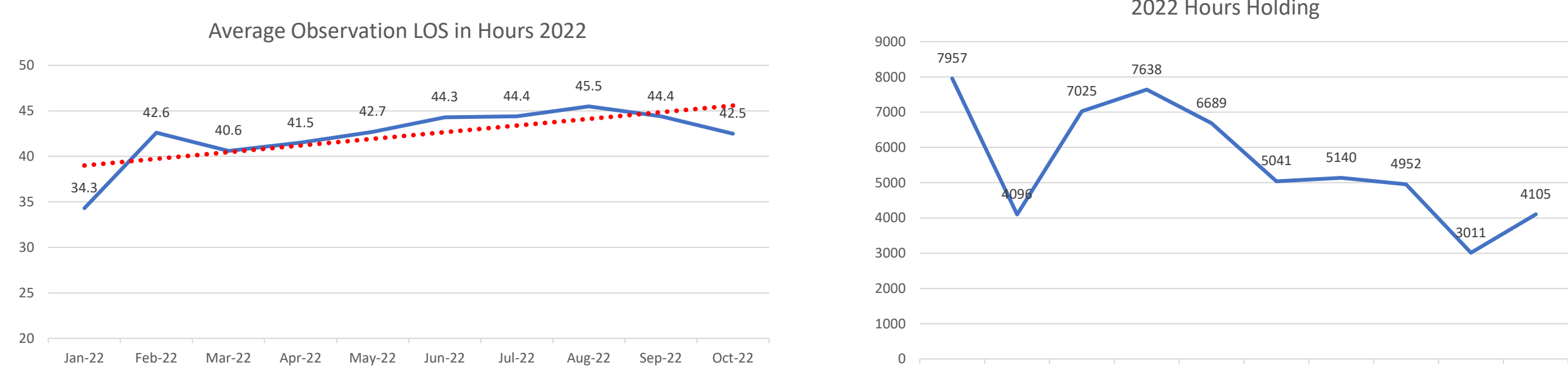


AIM

Focused action plan to decrease observation patients' length of stay (LOS) supporting increased hospital bed capacity. The goal is for decision to discharge as observation or admission occurring under 23 hours of placement. This reduction in LOS will result in expanded site capacity, decreasing holding hours for all admitted and observation patients bed placement. Observation LOS at other observation units at Wyandotte Hospital from October 2022-November 2023 average is at 48.14 hours vs. CDU Observation LOS at 16.35 hours. These actions will support nursing/provider satisfaction, increase Emergency Department (ED) capacity to greater serve our community and facilitate more time, safe, efficient and equitable care for our patients.

PLAN: CURRENT STATE

- Average observation hours significantly over the 23-hour allotment
- Increased ED holding hours and LWSC/AMA



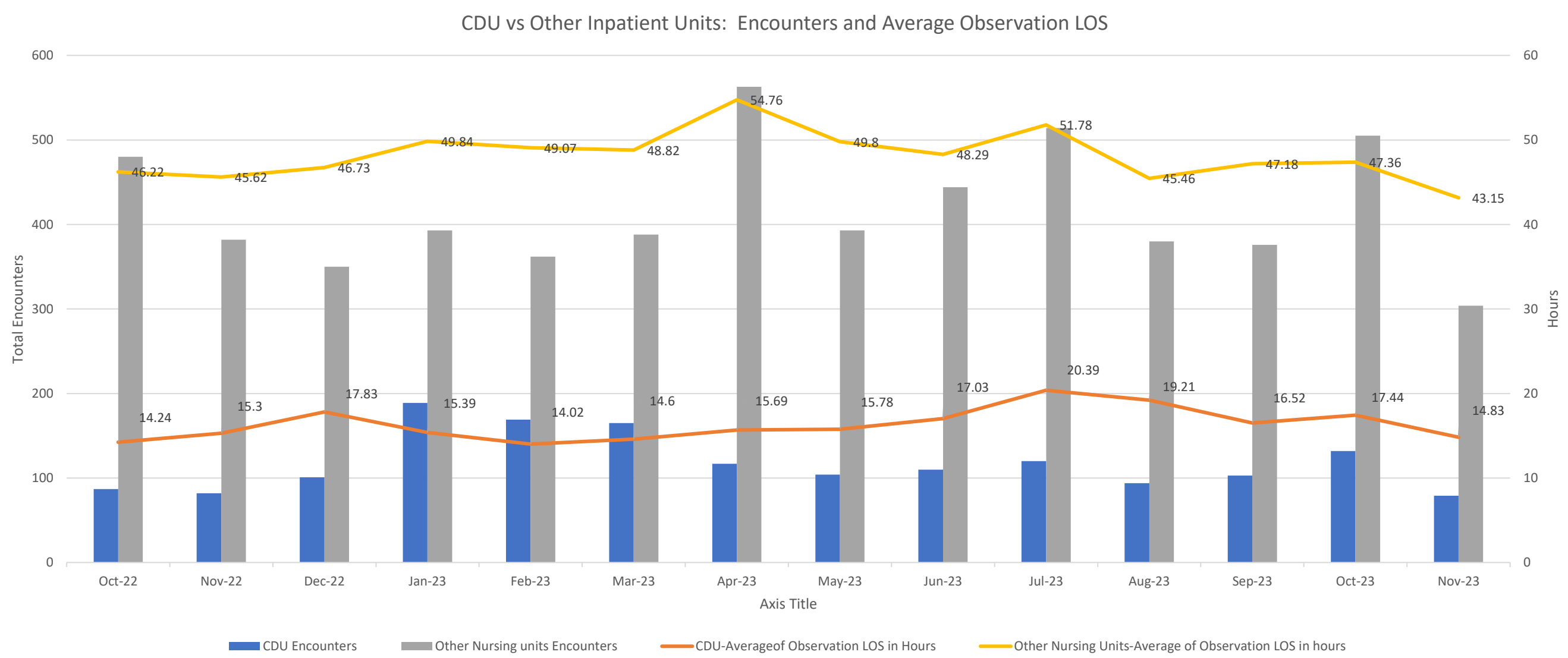
DO: CORRECTIVE ACTIONS / INTERVENTIONS

- Allocated 10-bed unit with dedicated staff and provider
- Coordination with ancillary and support units for priority testing and results
- Developed specific inclusion/exclusion criteria and pathway diagnoses for unit placement
- Created workflows for placement into the unit
- Provided education to ED providers for process and placement
- CDU provider with specific guidelines for treatment

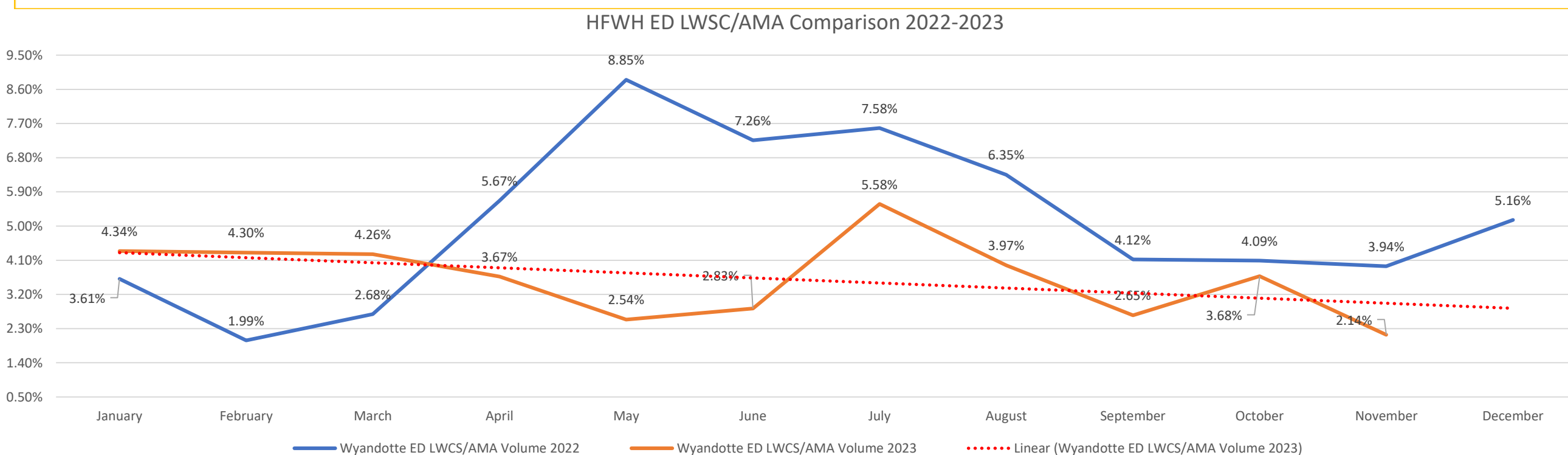
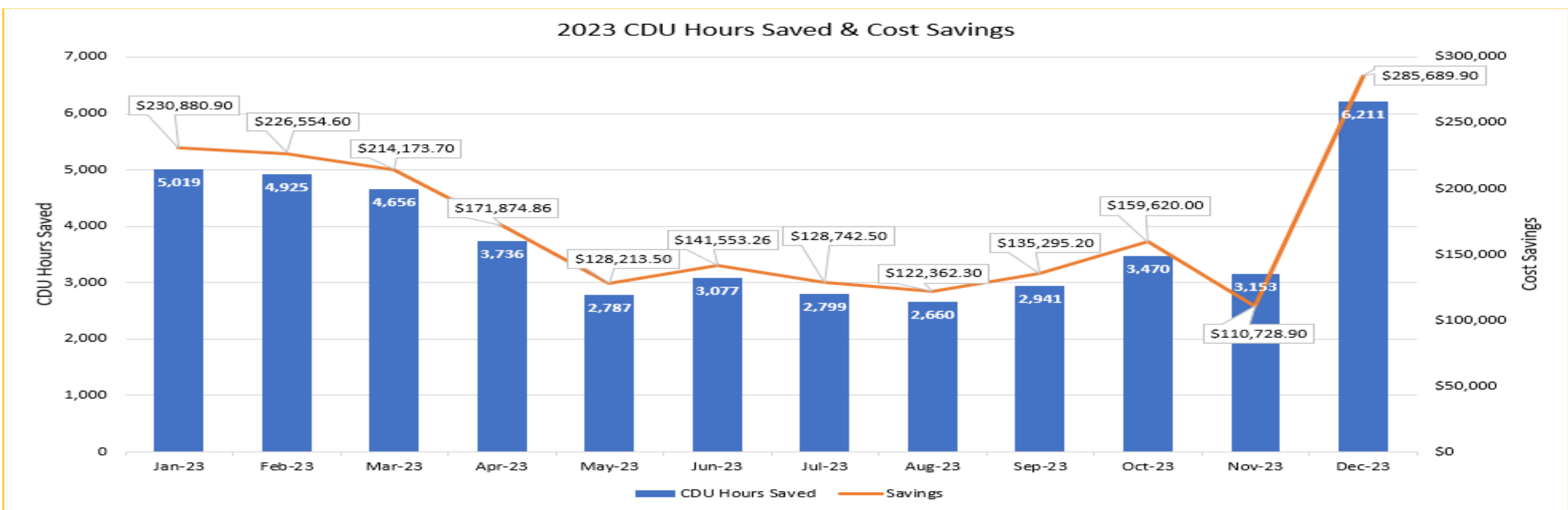
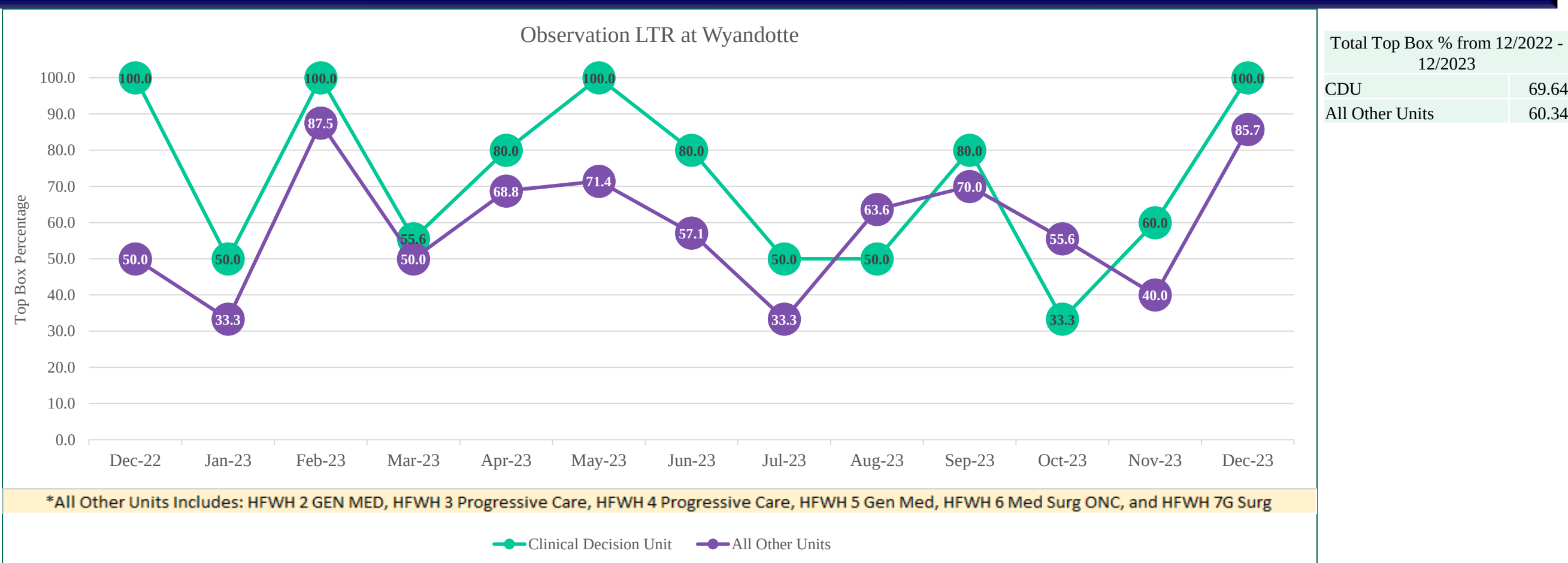
CHECK (EVALUATION OF CHANGES)

- Daily leadership rounding in unit
- Bi-monthly team meetings to discuss improvements, process changes, data
- Direct input from CDU provider for process improvements
- Continuous review of patients that meet criteria for the CDU and review of patients that met CDU criteria, but were not placed in unit
- Expansion of diagnoses to increase volume in the CDU
- Decision to open CDU to Brownstown ED and Fairlane ED for patient appropriate transfers

Comparison of observation hours of the CDU and other nursing units in hospital



MEASURES

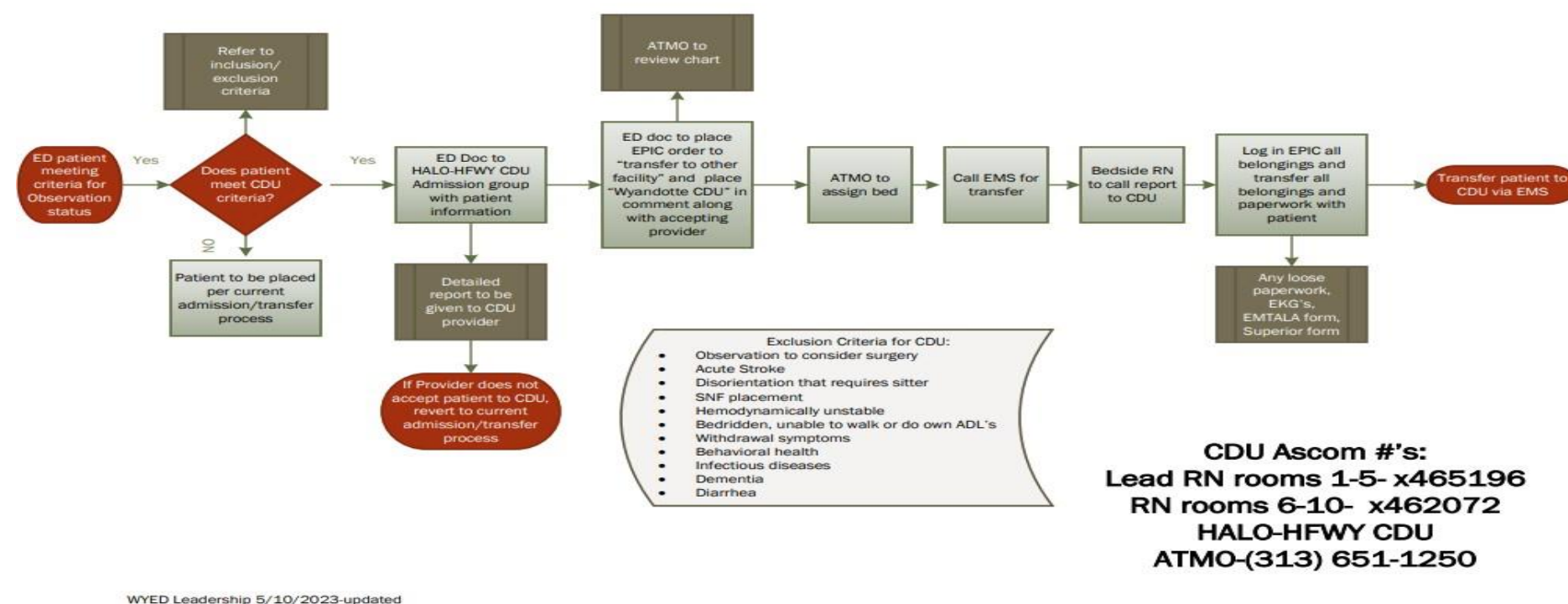


ACT: SUSTAIN AND SPREAD

- Continue with bi-weekly meetings with team for process sustainment and improvements
- Efficiencies gained are shared with site leadership to spread to all observation through the hospital
- CDU unit staffed and open 24 hrs./day 7days/week

Workflow for free standing ED's

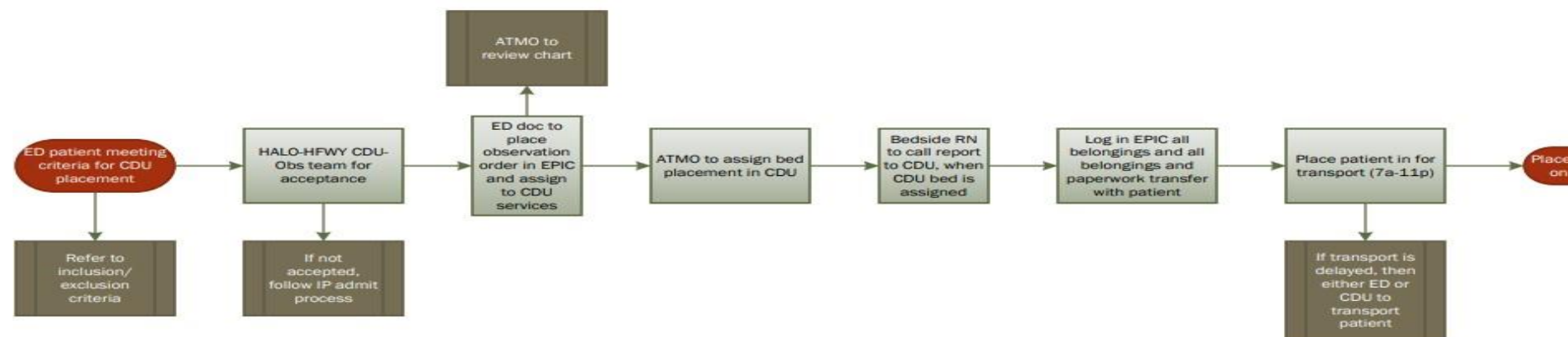
ED to CDU Workflow
Free standing ED's



CDU Ascom #'s:
Lead RN rooms 1-5 x465196
RN rooms 6-10 x462072
HALO-HFWY CDU
ATMO-(313) 651-1250

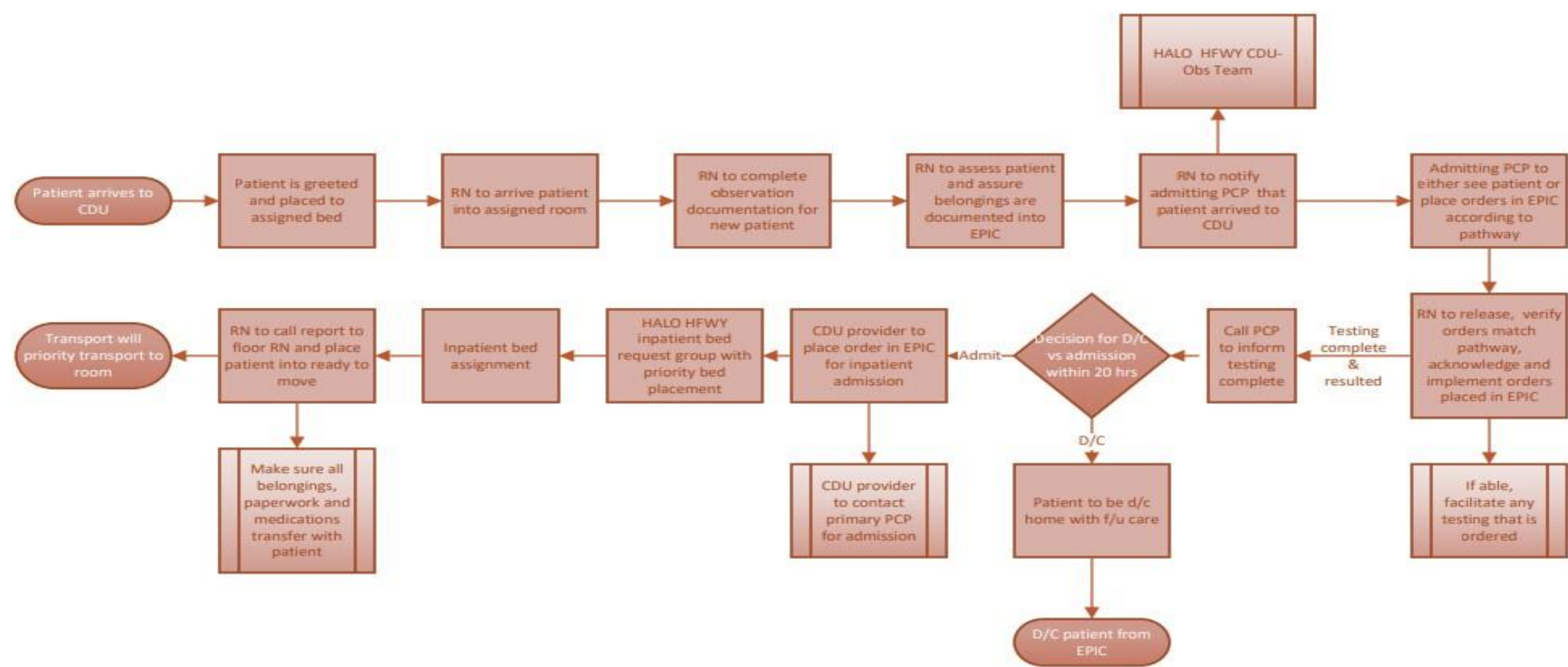
Workflows

ED to CDU Workflow



CDU Ascom #'s:
Lead RN rooms 1-5
x465196
RN rooms 6-10 x462072

CDU Workflow



KEYS TO SUCCESS / LESSONS LEARNED

- Dedicated staff and provider for unit
- Timely test results and frequent communication with patients, staff and provider
- Continuous review of data and process improvement
- Work closely with ATMO for expediting placement

Exclusion Criteria for CDU:

- Observation to consider surgery
- Acute Stroke
- Disorientation that requires sitter
- SNF placement
- Hemodynamically unstable
- Bedridden, unable to walk or do own ADL's
- Withdrawal symptoms
- Behavioral health
- Infectious diseases
- Dementia
- Diarrhea

Diagnoses

- Uncomplicated Chest pain
- Abdominal Pain
- Asthma
- COPD
- Cellulitis
- Syncope
- TIA